June 16, 2025

Centers for Medicare and Medicaid Services
7500 Security Blvd
Baltimore, MD 21244

RE: Request for Information; Health Technology Ecosystem (RFI)

Submitted electronically via www.regulations.gov

National Patient Advocate Foundation (NPAF) appreciates the opportunity to submit comments on this RFI on behalf of the patients we serve.

Background

NPAF advocates for policies that are important to patients and caregivers in healthcare reform, including specific steps to preserve their financial and social stability alongside their medical and mental health in the context of coping with complex or chronic conditions.

For most patients and families, financial distress is a common consequence of illness. This is true even for individuals with health insurance. Costs of medical and other care escalate fast and often overwhelm even the best planned household budgets. This hurts individuals' economies, access, health, and quality of life. It also interferes with efficient and quality care delivery across health systems and government-sponsored insurance and safety net programs.

NPAF is the advocacy affiliate of Patient Advocate Foundation (PAF), our direct patient services counterpart that delivers a skilled needs navigation model specifically supporting social and financial well-being for thousands of limited-resourced patients and families across the nation.

Needs Navigation aims to ease healthcare costs concerns that are a common condition across patient populations. P*rovided hands-on by individuals skilled in communication and resources coordination, Needs Navigation helps secure access to safety net services and other responsive assistance personalized to preserve patient and family financial health.*

NPAF is working to make Needs Navigation available in every care setting as a care model available to address patients' need to make ends meet and maintain their financial health while coping with illness. A 2025 Gallup poll supports the need for this approach, finding that more than one-third of Americans (35%) report that they are unable to access quality, affordable healthcare, four points higher than in 2023 and a new high since 2021.

This needs navigation model should now be expanded and integrated across all Medicare programs to provide real-time relief and practical help for limited-resourced patients and families confronted with social issues, emotional ones, medical debt, household financial hardships, or other financial and social strains that contribute to poorer health outcomes.

With this background, here are our recommendations for this RFI:

Recognizing and addressing financial hardship

In addition to the above Gallup data, a large majority, 73%, of Americans reported financial stress in a recent national survey. This is even higher for patients and family caregivers. A 2025 national survey of 2,510 patients and caregivers fielded by our Patient Insight Institute found 82% of them experienced financial difficulties in the past year because of expenditures related to medical care, insurance, cost of living challenges, lost wages, and more.

The evidence confirms that such financial difficulties can worsen health outcomes and result in additional health spending. Therefore, any health technology initiatives must account for and recognize access and other challenges associated with patients' financial needs and hardships. Assessing for unmet needs should be normalized and systematized as part of healthcare, which can be supported with technology and apps to supplement hands-on assistance of human beings skilled in Needs Navigation to assess and address financial risks and their impact on care.

Correcting inaccurate information in health records

We support the goal to "accelerate progress towards a patient-centric learning health system". A major issue we have heard from patients has to do with correcting inaccurate information entered in the electronic health record. Although HIPAA includes the right of patients to do so, many find that actual process very unclear and complicated, if not impossible. They've described the challenges of having incorrect diagnoses and/or stigmatizing mental or behavioral health issues following them forever, despite multiple efforts to make corrections with the help of their providers. This causes distress, inefficiencies in care, and even additional errors as providers make assumptions about patients that are incorrect and billing practices

follow suit. For example, inaccurate EHR information indicating "substance use disorder" positions providers to potentially overlook biological causes for a patient's physical or psychological symptoms, and even worse, disregard the patient's complaints about those symptoms. As opportunities evolve to improve health technology and its use in the healthcare ecosystem, it is important to include thoughtful consideration to develop seamless processes that are clear and accessible for patients and providers to easily correct information that is incorrect in their charts.

Conclusion

NPAF appreciates the opportunity to comment on this proposed regulation. Engaging patients in NPAF's policy development practices and providing a needs navigation model directly to patients and caregivers is a hallmark of our 29 years of organizational experience, expertise, and history. We are happy to share lessons learned and welcome the opportunity to meet with agency staff to discuss these recommendations to scale needs navigation and skilled clinical communication in all efforts to achieve healthcare reform that helps the system work for everyone. Please contact me at Rebecca.kirch@npaf.org if NPAF can provide further details.

Respectfully submitted,

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